Fax cover sheet

Canyon Crest Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Denise Whitfield, RN CM
Phone: 555-0170
Date: 08/20/2026 14:15
Pages (including cover): 13
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

41M R forearm cellulitis, improved, on PO antibiotics.
Medically ready for d/c. Requesting recuperative care.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH • HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Canyon Crest Hospital (fictional) Dept/Floor: 3 West Med-Surg
SW/CM/RN: Denise Whitfield, RN CM Phone/Pager #: 555-0170
Email: dwhitfield@canyoncrest.example Nursing Station #: 555-0171
Authorized by: Dr. L. Moreau Phone #:

Patient Information

Patient Name: Marcus Doe MRN: CC-0934178 Date of Birth (dd/mm/yyy): 05/30/1985
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: Black/African American
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Cellulitis R forearm
Admit Date: 08/16/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 08/16/2026 Results: CXR clear

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* ☒ No	Active Infection	☒ Yes* No
*Explain		*Explain	cellulitis, PO abx
Self Administer All Meds	☒ Yes No*	Ambulatory	☒ Yes No
Continent of Bowel & Bladder	☒ Yes No	Assistive Device	Yes* ☒ No
Colostomy/Ileostomy	Yes ☒ No	*Explain	
Foley Catheter	Yes ☒ No	Hospice	Yes* ☒ No
Diabetes	Yes ☒ No	*Explain	
Insulin	Yes ☒ No	Car	Yes ☒ No
Diabetes related Oral Meds	Yes ☒ No	Service Animal	Yes ☒ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? No Yes

☒ Mental Health DX If checked, what type?
☒ Non-compliant ☒ Forgetful
☒ Cognitive Impairment ☒ Registered Sex Offender
☐ Other Explain

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Marcus Doe

DOB: 05/30/1985

Member ID: OCH-82750146

Referring Provider

Facility: Canyon Crest Hospital (fictional)

Clinical

Primary Diagnosis: Cellulitis of right upper limb

ICD-10: L03.113

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 08/20/2026

Requested Days: 14

Plan Use Only

Status: Pending review

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Patient face sheet

Canyon Crest Hospital (fictional) - PATIENT FACE SHEET

Patient: Marcus Doe
MRN: CC-0934178
Account #: 99934178
DOB: 05/30/1985
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 08/16/2026 18:30
Admit Type: Inpatient
Unit/Bed: 3W-316B
Attending: Lucien Moreau, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical - page 1

HISTORY AND PHYSICAL

Date: 08/16/2026

Attending: Lucien Moreau, MD (Hospital Medicine)

CC

Right forearm redness and swelling x 3 days

HPI

41 y/o M experiencing homelessness (sleeps at a bus shelter near Example Ave) presents with 3 days of spreading erythema and pain of the R forearm after a scratch while moving pallets. Fever at home per pt. Denies IVDU. In ED: T 100.9F, HR 104, WBC 13.6. Bedside US: no abscess.

PMH

- Depression (per pt, untreated x 2 years)
- Asthma, mild intermittent

Meds PTA

- None

Allergies

NKDA

Social

Homeless x 1 year after job loss. Tobacco none. EtOH: 2-3 beers on weekends, last drink 5 days ago. Denies drugs.

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History and Physical - page 2

Exam

T 100.6F HR 100 BP 130/82 RR 18 SpO2 98% RA
Gen: NAD, AOx4
Skin: R volar forearm erythema 12 x 7 cm, warm, tender, no fluctuance, no lymphangitic streaking
Psych: flat affect, limited eye contact

Assessment / Plan

41M with non-purulent cellulitis R forearm.
1. Cellulitis - cefazolin 2 g IV q8h, elevate, mark borders; blood cx x2
2. MRSA nares PCR, COVID per protocol
3. Depression, untreated - PHQ-9 by nursing; psychiatry consult if positive screen
4. Dispo - CM consult; homeless, possible recuperative care

DVT ppx: enoxaparin 40 mg SC daily
Code: Full
Signed: Lucien Moreau, MD 08/16/2026 22:05

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Nursing note

NURSING NOTE

Date: 08/17/2026 15:40

PHQ-9 completed: score 22 (severe). Item 9 (thoughts of being better off dead or hurting yourself): "nearly every day."

Columbia (C-SSRS) screen positive - high risk.

Pt states he has been thinking about ending his life and has thought about how he would do it.

Charge RN and Dr. Moreau notified 15:45. 1:1 sitter initiated 15:50. Room safety sweep completed, belongings searched per policy.

Psychiatry consult ordered STAT.

Signed: B. Osei, RN

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Psychiatry consultation

PSYCHIATRY CONSULTATION

Date: 08/17/2026 19:20

Consultant: Ingrid Halvorsen, MD (Psychiatry)

Reason: Positive suicide screen

History

41M with long-standing untreated depression, worsened since losing housing. Reports low mood, poor sleep, hopelessness x 2 months.

Suicide Risk Assessment

Patient endorses active suicidal ideation with a plan to overdose on pills he would buy after discharge. States intent is "maybe 50/50." No prior attempts. No firearms access. Denies homicidal ideation. Denies auditory or visual hallucinations.

Assessment

Major depressive disorder, single episode, severe, without psychotic features. High acute suicide risk.

Plan

- Continue 1:1 sitter
- Start sertraline 50 mg PO daily
- Not psychiatrically stable for discharge to an unsupervised setting
- Recommend inpatient psychiatric admission once medically cleared

Signed: Ingrid Halvorsen, MD 08/17/2026 20:10

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5

Date: 08/20/2026 08:05

S: Forearm pain 1/10. Eating well. Remains low in mood.

O: T 98.3F HR 80 BP 124/78 RR 16 SpO2 99% RA

R forearm erythema receding well inside marked borders, no fluctuance.

Blood cx 08/16 x2: no growth at 96h.

A/P:

- Cellulitis R forearm, improved. Transition to cephalexin 500 mg PO QID to complete 7 days total (last dose 08/22).
- Medically stable for discharge.
- MDD with suicidal ideation - psychiatry following, 1:1 sitter continues.

Signed: Lucien Moreau, MD

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Psychiatry follow-up note

PSYCHIATRY FOLLOW-UP NOTE

Date: 08/20/2026 10:30

Pt seen with sitter at bedside. Mood "the same." Continues to endorse suicidal ideation with the same plan to overdose; intent unchanged. Tolerating sertraline day 3.

No hallucinations. Calm, cooperative, no aggression.

Assessment: MDD severe, ongoing high suicide risk.

Plan: Continue 1:1. Recommend transfer to inpatient psychiatry when bed available. Not appropriate for an unsupervised or low-supervision setting at this time.

Signed: Ingrid Halvorsen, MD

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Physical therapy screen

PHYSICAL THERAPY SCREEN

Date: 08/18/2026 11:15
Bed mobility, transfers, gait: Independent
Gait: 300+ ft without device, independent, steady
ADLs per nursing: independent grooming, dressing, toileting, showering
No skilled PT needs. Sign off.
Signed: M. Delacroix, PT

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Vitals and labs

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
08/19 08:00	98.6	84	126/78	16	98%	RA
08/19 16:00	98.4	82	122/76	16	99%	RA
08/20 00:00	98.1	76	120/74	14	98%	RA
08/20 04:00	98.2	78	120/76	16	99%	RA
08/20 08:05	98.3	80	124/78	16	99%	RA

LABS

08/16 | WBC 13.6 H | Hgb 14.8 | Plt 276 | Cr 1.0
08/19 | WBC 8.1 | Hgb 14.1 | Cr 0.9 | Glu 92

MICROBIOLOGY

08/16 Blood cx x2: no growth at 96h
08/17 MRSA nares PCR: Negative
08/20 06:00 SARS-CoV-2 rapid antigen: Negative

IMAGING

08/16 CXR: clear lungs

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Discharge medication list

DISCHARGE MEDICATION LIST (PENDING)

Patient: Marcus Doe MRN: CC-0934178

- Cephalexin 500 mg PO QID through 08/22/2026
- Sertraline 50 mg PO daily (NEW)
- Albuterol HFA 90 mcg 2 puffs q4-6h PRN wheeze
- Acetaminophen 650 mg PO q6h PRN pain

Stopped

- Cefazolin IV, enoxaparin (inpatient only)

Patient able to self-administer all medications: Yes

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